

UID: E74C083F-6261-4C59-8A1F-6DD5975637D2
TCGA-L6-A4EQ-01A-PR

Redacted

Gender: [REDACTED]

Birth Date: [REDACTED]

Admit Protocol: [REDACTED]



Final Results

AP Supplemental Report

Final

CASE NUMBER: [REDACTED]
SUPPLEMENTAL

SUPPLEMENTAL

REPORT

REASON FOR THE SUPPLEMENTAL REPORT: To clarify the location of the lesions. Diagnosis remains unchanged.

DIAGNOSIS:

1. Lymph node, delphan lymph node, biopsy: Metastatic carcinoma to lymph node (1/1).
 2. Thyroid, total thyroidectomy:
 - Papillary thyroid carcinoma, conventional type, (3.5cm), isthmus
 - Hurthle cell neoplasm, favor Hurthle cell adenoma. (left lobe).
- See note:
- Multinodular goiter

ICB-0-3
carcinoma, papillary, thyroid 826 of 3
Site: thyroid, NOS C73.9

The original report is as follows:

DIAGNOSIS:

1. Lymph node, delphan lymph node, biopsy: Metastatic carcinoma to lymph node (1/1).
2. Thyroid, total thyroidectomy:
 - i. Papillary thyroid carcinoma, conventional type, (3.5cm), left lobe.
 - ii. Hurthle cell neoplasm, favor Hurthle cell adenoma. (isthmus). See note:
 - iii. Multinodular goiter.

NOTE: The Hurthle cell neoplasm shows no evidence of fibrous capsule and capsular or vascular invasion. This lesion stains positive for thyroglobulin and negative for Calcitonin.

Dr. [REDACTED] is informed by Dr. [REDACTED] of presence of micrometastasis in lymph node on [REDACTED]

Immunoperoxidase and in-situ hybridization tests performed here and used for diagnosis were developed and their performance characteristics determined by the Laboratory of Pathology, [REDACTED]. They have not been cleared or approved by the U.S. Food and Drug Administration. The FDA has also determined that such clearance or approval is not necessary. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA) as qualified to perform high complexity clinical laboratory testing.

[REDACTED]		Admit Protocol: [REDACTED]
Gender: [REDACTED]	Birth Date: [REDACTED]	Age: [REDACTED]
Admit Protocol: [REDACTED]	Admit Protocol: [REDACTED]	

Final Results

Clinical History: thyroid
cancer, papillary Specimen Taken For
Protocol: 01 - Yes

PROCEDURE: Pre-Operative Diagnosis: papillary thyroid ca
Post-Operative
Diagnosis: same Operative Findings: Isthmus nodule,
left lobe nodule

SPECIMENS SUBMITTED: 1. SERIAL 5 + 3FS, Delphan lymph node (1FS)
2. THYROIDECTOMY, TOTAL

INTRAOPERATIVE CONSULTATION:

1FS: "Delphan lymph node, rule out metastatic thyroid cancer".
1FS Diagnosis: Lymph node with fibrotic tissue, no tumor seen.

-The frozen section is performed by Dr. [REDACTED] and Dr. [REDACTED].
- Dr. [REDACTED] is informed by Dr. [REDACTED] of presence of micrometastasis in
lymph node on [REDACTED].

GROSS DESCRIPTION: Two specimens are received fresh with the
patient's name, medical record number, and further specified as follows:

1. "Delphan lymph node, rule out metastatic thyroid cancer". It
consists of a yellow-tan soft tissue fragment measuring 0.8 x 0.7 x 0.4
cm. Entirely frozen. In Surgical Pathology, the specimen matched the
above description and the specimen is transferred to cassette
[REDACTED].
2. "Total thyroid gland, long stitch right superior, short left
superior". It is a total thyroidectomy weighing 42.7 grams (post
procurement). Gross photographs are taken. The right lobe measures 3 x
2.5 x 1.5 cm. The left lobe 6.5 x 3 x 1.5 cm. Isthmus measures 4 x 4 x
1.5 cm. A solid lesion measuring 3.5 x 3.2 x 2.2 is identified in the
left lobe, while a cystic lesion 3.3 x 2.5 x 2 cm in deflated state is
identified in isthmus. Approximately 1 x 1 cm of tissue is procured for
Dr. [REDACTED] from each lesions. Procurement was performed by Dr.
[REDACTED] in OR and [REDACTED] on [REDACTED].
Also 0.5 cm of each tumor and 0.5 x 1 x 1 cm of normal tissue in the
right lobe is also procured for Dr. [REDACTED]. The right anterior
aspect of the gland (right lobe and isthmus) is inked blue, the left
anterior aspect of the gland (left lobe) is inked red, and posterior
aspect of the gland is inked black. Approximately 0.8 x 0.6 x 0.3 cm of
normal-appearing thyroid tissue from the right lobe is procured in TPPF
for the normal tissue bank by [REDACTED] on [REDACTED]
a.m. The remainder of the specimen is placed in formalin and submitted
for Pathology for permanent processing. In Surgical Pathology, the
specimen matched the above description and is entirely submitted. The
external capsule of the gland is grossly uninvolved by aforementioned
two lesions. The superior aspect of left lobe is grossly uninvolved
with a beefy-red cut surface and is submitted in cassettes 2A and 2B

Clinical Center

Gender: [REDACTED]

Birth Date: [REDACTED]

Admit Protocol: [REDACTED]

Age: [REDACTED] y

Admit Protocol: [REDACTED]

Final Results

from superior to inferior. The solid lesion in the inferior aspect of left lobe is yellow-tan and well-circumscribed with multiple focal punctate hemorrhage and multiple nodular lesion measuring around 0.5 x 0.6 cm but no necrosis or depressed scar. The solid lesion is submitted from left upper aspect to left lower aspect in cassette 2C-2J and right upper to right lower aspect in 2K-2O. The cystic lesion in the isthmus is gray-tan with a finely papillary and granular appearance and focal punctate hemorrhage. A calcified spot around 0.8x 0.6x 0.6 cm is identified in the right middle aspect of the cystic lesion. The isthmus is submitted from left upper aspect to left lower aspect in 2P-2R and right upper to right lower in [REDACTED] right aspect of isthmus and left aspect of right lobe from left to right. [REDACTED]: right lobe from superior to inferior. 2GG: superior aspect of very right end of right lobe. 2HH: inferior aspect of the very right end of right lobe.

No consultants

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Criteria	Yes	No
Diagnosis Discrepancy		/
Primary Tumor Site Discrepancy		/
HIPAA Discrepancy		/
Prior Malignancy History		/
Dual/Synchronous Primary Noted		/
Case is (circle): QUALIFIED / QUALIFIED		/
Reviewer Initials		

Confidential Patient Information. Unauthorized disclosure is prohibited by the Federal Privacy Act of 1974.

Requested By: [REDACTED] (Other)

Do not file in Medical Record